

**“AN OVERVIEW OF THE REGULATORY FUNCTIONS OF THE PHARMACISTS
COUNCIL OF NIGERIA: INSPECTORATE ACTIVITIES”**

A Presentation by

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1.0 Introduction

If one were to summarize the primary activity of the Registry staff of the Pharmacists Council of Nigeria (PCN), it would be **inspection and accreditation**:

- Inspection of pharmaceutical premises where drugs and medical devices are manufactured, stored, distributed, and/or offered for sale to the public.
- Accreditation of the Bachelor of Pharmacy (B.Pharm) programme in approved universities (Schools of Pharmacy), as well as pharmacy technician training in Schools of Health Technology.

For the PCN to effectively discharge its regulatory and control functions, pharmaceutical premises must be **registered, inspected, approved, and licensed**. Such premises cannot be considered licensed unless they are under the supervision of a pharmacist who has graduated from an accredited School of Pharmacy in Nigeria—or its recognized equivalent abroad—and designated as the superintendent pharmacist.

This paper expands on the regulatory functions of the PCN with particular focus on **Inspectorate activities**, as they relate to pharmaceutical premises involved in production, storage, distribution, dispensing, and sale of medicines and medical devices.

The process from a pharmacist's intention to establish a practice to the stage of its licensure by the PCN is detailed and rigorous. It involves key actors (inspectors), activities (inspections), objects (products), and facilities (premises).

The **National Drug Policy (NDP, 2005)** underscores this when it states that *“the effective inspection of drug products and manufacturing facilities is critical to ensuring the quality, safety, and efficacy of drugs.”* Within this regulatory framework lies the scope of inspectorate activities of the PCN.

2.0 Conceptual Clarification

2.1 Who is a Pharmaceutical Inspector?

A Pharmaceutical Inspector is a registered pharmacist who has undergone organized, approved training, and is formally designated by the PCN (Brown, 1999). According to the **World Health Organization (WHO, 2007)**, inspectors should normally be pharmacists with experience in community and/or hospital pharmacy.

2.2 Attributes of an Inspector (WHO, 2007)

An inspector must possess:

- Strong knowledge of pharmacy, drugs, and poisons.
- Sound understanding of relevant laws and regulations.
- Command of technical terms and excellent communication skills.
- Ability to detect forged or falsified documents.
- Maturity, honesty, and integrity.
- Responsible conduct that commands respect.
- Capacity to organize work with minimal supervision.
- Ability to make quick, rational, and sound decisions.
- Strong interpersonal skills with firmness and fairness.
- Ability to hold post-inspection discussions with management.
- Motivation, stamina, and commitment to long hours.
- Ethical conduct, avoiding conflicts of interest.

2.3 Definition of Pharmaceutical Inspection

Pharmaceutical inspection is a **planned system of activities** aimed at ensuring that drugs—whether manufactured locally, imported, dispensed in health institutions, or sold in pharmacies—are of acceptable quality, safe, and efficacious for diagnosis, prevention, and treatment of diseases in humans and animals (Brown, 2008).

In essence, it is the activity undertaken by officially designated PCN inspectors to ensure that practice in licensed premises conforms to **established standards**.

Key activities include:

- a) Inspection of drug manufacturing industries.
- b) Inspection of imported drugs at ports of entry.
- c) Inspection of pharmaceutical outlets (wholesale, retail, importation).
- d) Inspection of hospital pharmacies (PCN, 2009).
- e) Inspection of the pharmaceutical distribution chain by the **Drug Distribution Inspection Committee (DDIC)**, since July 2008.

3.0 Inspectorate Activities of the PCN

3.1 General Scope

The primary purpose of inspection is to ensure that the superintendent pharmacist and other staff of licensed premises deliver appropriate pharmaceutical care to the public.

Inspection is not limited to initial licensing—it must also be **periodic**, ensuring compliance with minimum standards and ethical practice.

The **Federal Republic of Nigeria Official Gazette No. 79, Vol. 92 (2005)** empowers inspectors to:

- Enter premises where pharmaceutical products are manufactured, stored, packaged, or sold.
- Examine articles, records, and facilities.
- Collect samples or specimens for analysis.
- Open and examine containers or packages.
- Review documents and records relevant to compliance.
- Seize and detain articles suspected of contravening regulations.

3.2 Retail/Wholesale/Distribution/Importation Premises

These outlets provide the public with access to medicines, poisons, and devices. Inspections are conducted under a **Public–Private Partnership (PPP)** framework, with pharmacists of proven integrity co-opted into the process.

The PCN's *Manual for Pharmaceutical Inspectors in Nigeria* reminds inspectors that “*to whom much power is given, much responsibility is expected.*” Inspections should not be limited to identifying faults, but should be an opportunity to assist, guide, and motivate practitioners toward compliance with **Good Manufacturing Practice (GMP)** and **Good Pharmacy Practice (GPP)**.

3.3 Hospital Pharmacies

Hospital pharmacies operate within health institutions, primarily to fill prescriptions and provide services such as compounding, drug information, and formulary management. The **Registration of Pharmaceutical Premises Regulations (2008)** mandates that every hospital pharmacy be registered with the PCN.

The **PCN's Compendium of Minimum Standards** specifies staffing requirements based on bed capacity at primary, secondary, and tertiary healthcare levels. These standards serve as benchmarks during inspection.

In 2009, the PCN convened a workshop to develop consensus statements on hospital pharmacy practice in Nigeria, aligning with global trends to shift pharmacy's focus from *products* to *patients*.

3.4 Pharmaceutical Manufacturing

The focus here is compliance with **current Good Manufacturing Practice (cGMP)**. Inspection of manufacturing plants requires specialized expertise, as it covers diverse process areas.

Between 2004 and 2009, the PCN invested significantly in training its inspectors—both locally and internationally—ensuring they acquired the skills necessary to evaluate pharmaceutical plants to the highest standards.

Table 1: Major Efforts in the Training and Development of PCN Pharmacists in Inspection of Pharmaceutical Industries

S/No	ACTIVITIES	PERIOD	Venue	Remarks
1.	Meeting with Superintendent and Production Pharmacists of Pharmaceutical Manufacturing Industries in Nigeria.	October 16, 2004	Airport Hotel Ikeja, Lagos	
2.	PCN Factory Inspection Training by Nigerian German Chemicals (NGC) for Superintendent and Production Pharmacists and registry Staff	September 26-20, 2005	Lagos Office, NGC Premises Lagos	Pharm. Yomi Osewa facilitated
3.	Workshop for manufacturer and Importers of Pharmaceutical Products towards evolving a systematic distribution system	August, 2006	Sheraton Hotel, Abuja	The Honourable Minister of Health, Prof. Eytayo Lambo attended as Special Guest of Honour
4.	Training on Fundamental plus essential of validation and Pharmaceutical water system by Rodot consulting for Registry Staff	December 2006	Sheraton Hotel, Ikeja, Lagos	For staff of PCN and NAFDAC
5.	Good Manufacturing Practice (GMP training for PCN Inspectors	July 19 - 20, 2008	PCN Head Office/Registry Idu, Abuja	Pharm. Yomi Osewa facilitated
6	WORKSHOP FR TEN (10) Staff of the PCN by the World Health Organization (WHO) on Good Pharmaceutical Distribution	June 22- 25, 2009	Otta, Ogun State	22 People were in attendance, 10 pharmacist from PCN, 7 Pharmacists from NAFAC, 3 Chemists and Biochemists and

				3 Consultant from WHO
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7	Training for Pharmaceutical Inspectors Staff of PCN on aspect of Current Good Manufacturing Practice (cGMP)	Lagos (July 13 th)	Multipurpose Centre	88 Pharmacists staff of PCN attended. Pharm. Yomi and Dr. Echeazu Ogu were trainers
		Abuja (July 23 rd)	Sheraton Hotel	
		Enugu (July 27 th 2009)	Nike Lake Hotel	
8	Two days advocacy workshop for CEO's superintend Pharmacists in Nigeria's Pharmaceutical Manufacturing Industries on c GMP	Lagos (July 14 th - 15 th , 2009))	Multipurpose Centre	Total of 29 CEOs, 134 Superintendent Pharmacists and 11 Production Pharmacists attended.
		Abuja (July 21 st - 23 rd)	Sheraton Hotel	
		Enugu (July 27 th - 29 th ,2009)	Nike Lake Hotel	
9	The Deputy Director, Inspection and Monitoring participated in a workshop on "United Nations Prequalification of Medicines, Diagnostic and Vaccines" at the 6 th Consultative Stakeholders Meeting with manufactures on the invitation of NAFDAC	April 4-5, 2011	Geneva switzerland	

3.4 Drug Distribution

Drug distribution as a process requires close inspection. Any properly manufactured or imported pharmaceutical product registered with **NAFDAC** must only be distributed within premises that have been **inspected, approved, registered, and licensed** by the PCN.

The **National Drug Policy (NDP, 2005)** provides explicit guidance on rational drug distribution:

- i) Drug distribution, supply, sale, and dispensing shall be under the control and supervision of pharmacists at all levels.
- ii) Government shall ensure that drug manufacturing, wholesaling, and retailing activities are registered as distinct enterprises.
- iii) In the private sector, the distribution channel shall flow from manufacturers/importers → wholesalers → retailers.
- iv) Supplies to public health facilities shall be based on expressed need, primarily through the Central Medical Stores.
- v) All drugs purchased or donated to government at any level must be channeled through the Central Medical Stores.
- vi) Hospitals and clinics shall maintain inventory control systems, including computerized records linked to the Central Medical Stores.
- vii) Adequate security must be provided for storage areas, especially for narcotic drugs.
- viii) All drugs distributed in the country shall be labeled in English.
- ix) Incentives shall be created to encourage pharmacists to establish practices in rural areas. Professional associations and regulatory bodies should also support rational drug use and greater access to quality medicines.

In line with this, the **Drug Distribution Inspection Committee (DDIC)** was inaugurated on July 2, 2008. Its focus is safeguarding patients and the public by ensuring:

- Protection against malpractice by distributors and suppliers.
- Strict adherence to drug laws governing compounding, distribution, importation, exportation, and storage.
- Maintenance of high ethical and professional standards.

The **Federal Republic of Nigeria Official Gazette (No. 81, Vol. 92, 2005)** further mandates proper record-keeping across all levels of the pharmaceutical distribution chain, requiring manufacturers, importers, wholesalers, and retailers to keep accurate and auditable records of purchases, sales, prescriptions, production, and quality control.

To date, the DDIC has inspected **86 premises** (retail, wholesale, and manufacturing) across **13 States of the Federation**.

3.5 Patent and Proprietary Medicine Shops (PPMS)

The inspection of PPMS outlets is carried out nationwide by the **Patent and Proprietary Medicine Vendors Licence (PPMVL) Committees**, chaired by the Directors of Pharmaceutical Services (DPSs).

The approved drug list serves as a guide for the medicines permitted in these shops. However, numerous challenges persist, and inspectors remain familiar with these difficulties during field operations.

4.0 Challenges to Pharmaceutical Inspectors in Discharging Their Regulatory Functions

Pharmaceutical inspectors face many challenges, including:

- a) Lack of cooperation from owners or superintendent pharmacists of inspected premises.
- b) Risks to inspectors' lives and inadequate logistical support.
- c) Collusion between pharmacists and business operators.
- d) Insufficient logistics and funding.

5.0 Conclusion, Recommendations, and Implementation Strategies

5.1 Conclusion

As **Prof. Fola Tayo** emphasized in his keynote at the 4th Fred Adenika Memorial Lecture (2007):

"To perform effectively, National Regulatory Authorities (NRAs) must be provided with the necessary political support, legal powers, human and financial resources. However, these alone will not guarantee effective medicines regulation. NRAs also need strong public support and proper management."

Therefore, agencies such as the **PCN** and **NAFDAC** require both governmental and public support to effectively perform their regulatory roles. Medicines must be strictly regulated to:

- Address information gaps between manufacturers/sellers and patients/consumers.
- Protect desperate patients from ineffective or toxic medicines.
- Prevent misuse (e.g., antibiotics) that may harm individual and public health.
- Ensure all actors in the drug supply chain uphold treatment outcomes.

5.2 Recommendations and Implementation Strategies

Recommendation 1: Adequate Funding

The Federal Government, through the FMOH, should increase PCN's funding for inspections, recognizing that regulatory inspections are **not a revenue-generating venture**.

Implementation:

- PCN to prepare annual inspection budgets with clear timelines.
- FMOH to propose special budgetary allocations dedicated to inspection activities.
- Funds to be used for:
 - i. Procuring more vehicles for inspectors.
 - ii. Regular local and international training of inspectors to remain ahead of those they regulate.

Recommendation 2: Collaboration with the Police

PCN should partner with the Nigerian Police to strengthen inspections.

Implementation:

- IGP to direct State Commands to support PCN inspectors.
- Police officers to be sensitized on pharmacy laws to better understand their roles during inspections.

Recommendation 3: Judicial Enlightenment

The judiciary must be better informed about the role of inspectors in safeguarding public health.

Implementation:

- Organize periodic workshops for judges.
- Encourage judges to carefully evaluate injunctions related to pharmaceutical inspections.

Recommendation 4: Public and Professional Cooperation

Pharmacists and the public should see inspectors as partners, not adversaries.

Implementation:

- Revise pharmacy curricula and MCPD programmes to include detailed objectives of inspection as a growth tool.
- Reframe inspections as **corrective** rather than punitive.
- Conduct public awareness campaigns to explain the benefits of inspections.
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